REASON FOR VISIT: Subacute onset of fluctuating double vision (diplopia), bilateral droopy eyelids, and generalized muscle weakness that worsens toward the evening.

HISTORY: A 42-year-old female presents reporting progressive fatigue and visual disturbances over the past two months. She states that upon awakening in the morning, her vision is clear, but by late afternoon, she experiences horizontal diplopia and noticed her left eyelid drooping. Over the last three weeks, she has had difficulty chewing tough meats toward the end of dinner, and her voice becomes nasal after prolonged telephone conversations. She denies shortness of breath, orthopnea, or difficulty swallowing liquids.

NEUROLOGICAL EXAMINATION:

Cranial Nerves:

- Significant bilateral, asymmetric ptosis (left worse than right). Upward gaze sustained for 60 seconds produces worsening of the left ptosis (fatigability test positive).

- Ice Pack Test: Application of an ice pack over the left eye for 2 minutes produces immediate, dramatic elevation of the palpebral fissure by 4 mm (positive ice pack sign).

- Extraocular Movements: Incomplete abduction of the right eye and incomplete adduction of the left eye on horizontal saccades, which fluctuates during testing. Pupils are 4 mm, round, and briskly reactive to light (pupillary sparing).

- Orbicularis Oculi: Moderate weakness; examiner easily pries open forced eye closure.

- Bulbar Function: Mild nasal speech after reading aloud for two minutes. Palatal elevation intact. Normal tongue protrusion.

Motor Function:

- Neck flexion strength is 4/5; neck extension 5/5.

- Proximal muscle endurance: Repeated shoulder abduction demonstrates progressive fatigue from 5/5 down to 4-/5 after 20 repetitions; distal grip strength is intact.

Sensory & Reflexes: Sensation to all modalities intact throughout. Deep tendon reflexes 2+ symmetrically. Plantar responses flexor.

Respiratory: Forced vital capacity measured at bedside is 2.8 liters (88% of predicted value).

DIAGNOSTIC WORKUP:

1. Serum Acetylcholine Receptor (AChR) Binding Antibodies: Strongly positive at 14.8 nmol/L (reference < 0.5 nmol/L).

2. Repetitive Nerve Stimulation (RNS) Study: 3 Hz stimulation of the spinal accessory and facial nerves demonstrates a progressive, reproducible 18% decremental response in compound muscle action potential (CMAP) amplitude between the first and fourth potentials.

3. Contrast-Enhanced Chest CT: Excludes thymoma; demonstrates mild thymic follicular hyperplasia.

DIAGNOSIS AND TREATMENT:

1. Generalized Acetylcholine Receptor Antibody-Positive Myasthenia Gravis (MGFA Class IIb).

2. Initiate oral pyridostigmine bromide (Mestinon) 60 mg three times daily with meals.

3. Initiate low-dose oral prednisone 20 mg daily, titrating gradually to avoid paradoxical myasthenic weakness.

4. Referral to cardiothoracic surgery for elective minimally invasive robotic thymectomy.